

AI-Powered HMS Synthetic Clinical Dataset

Encounter Note

Patient Name	Trinh Kim Nga	MRN	MRN-0095
DOB	1980-01-14	Gender	Female
Encounter Date	2024-01-03	Department	Emergency Medicine
Attending Provider	Pharm. Riya Patel	Status	discharged

Synthetic Data Warning: This document is generated for software testing, OCR parsing, chunking, embedding, access-control validation, and Graph RAG demonstrations. It is not valid medical advice and must not be used for patient care.

Subjective

Trinh Kim Nga is a 46-year-old patient in Emergency Medicine presenting for follow-up of acute symptoms requiring triage and short observation. PMH includes episodic symptoms without persistent instability. Patient reports symptoms are stable overall. Denies severe chest pain, syncope, active bleeding, or new focal neurologic deficit unless otherwise documented. Current meds include acetaminophen 500mg QID PRN; ondansetron 4mg Q8H PRN. Allergies: latex sensitivity.

Objective

Vitals: BP 135/85 mmHg, HR 77 bpm, RR 21/min, Temp 36.8 C, SpO2 96% RA, weight 66 kg. Exam: alert, oriented, no acute distress. Cardiopulmonary exam without unstable findings. Abdomen soft, non-tender. Extremities show no acute deformity; edema varies by department-specific condition. Labs reviewed below; medication list reconciled.

Recent Lab Snapshot

Date	Analyte	Value	Unit	Reference Range	Status
2026-05-05	Creatinine	1.21	mg/dL	0.7-1.3	Normal
2026-05-05	eGFR	65.0	mL/min/1.73m2	>=60	Normal
2026-05-05	Glucose	110.0	mg/dL	70-99	High
2026-05-05	HbA1c	6.2	%	<5.7	High
2026-05-05	BNP	228.0	pg/mL	<100	High
2026-05-05	AST	39.0	U/L	10-40	Normal
2026-05-05	ALT	41.0	U/L	7-56	Normal
2026-05-05	Potassium	4.2	mmol/L	3.5-5.0	Normal
2026-05-05	Hemoglobin	13.5	g/dL	12.0-17.5	Normal

Assessment

Primary assessment: undifferentiated chest pain or acute dehydration, clinically stable after evaluation. Patient is clinically stable for ongoing management. Risk considerations include renal dosing, medication safety, fall risk, and need for timely escalation if red-flag symptoms occur.

Plan

Repeat vitals and ECG if symptoms recur. Encourage oral hydration. Provide return precautions for chest pain, syncope, or SpO2 < 92%. Provide patient education, update care team via SBAR, and document any access-relevant clinical justification before AI-assisted retrieval of PHI.

Detailed Care Coordination Notes

Problem List

1. undifferentiated chest pain or acute dehydration, clinically stable after evaluation. 2. Medication safety monitoring. 3. Functional status and discharge readiness. 4. Follow-up coordination with Emergency Medicine.

Safety Triggers

Escalate to responsible clinician for SBP < 90, HR > 130, SpO2 < 92%, acute mental status change, active bleeding, severe pain, fever with neutropenia risk, or rapidly rising Cr/K.

AI Retrieval Tags

access_tags: read, summary, labs, medication, cardiology as applicable. Suggested document_type for ingestion: encounter_note.